

SecureCare Plus Health Insurance Policy

Sample policy intentionally designed for Money Docs Decoded testing

Policy field	Value
Policy number	SCP-DEMO-2026-004817
Product	SecureCare Plus — Individual & Family Floater
Policy period	01 April 2026 – 31 March 2027
Sum insured	■10,00,000
Insurer	Example Health Assurance Ltd. (synthetic)
Premium	■28,750 including applicable taxes
Members	Self + spouse + one dependent child

Important Notice

This document is fictional and created solely for software testing. It contains deliberately varied policy clauses, exclusions, limits, waiting periods, co-payments and fees so that a document-analysis system can identify and cite them.

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1. Coverage and Limits

1.1 Hospitalisation

The Company will reimburse reasonable and medically necessary expenses incurred during an admissible hospitalisation, subject to the Sum Insured, applicable sub-limits, deductibles, co-payment, exclusions and other conditions of this Policy.

1.2 In-patient treatment

Coverage includes room charges, nursing charges, surgeon and physician fees, prescribed medicines, diagnostic investigations and eligible hospital consumables, subject to the limits described in this document.

1.3 Day-care treatment

Day-care procedures listed by the Company are covered when the procedure requires less than 24 hours of hospitalisation because of advances in medical technology. Procedures not appearing on the Company's current list may require prior confirmation.

1.4 Pre- and post-hospitalisation

Eligible medical expenses incurred up to 30 days before admission and 60 days after discharge will be considered, provided the expenses relate directly to the admissible hospitalisation.

1.5 Ambulance

Road ambulance expenses are covered up to ■3,000 per hospitalisation. Air ambulance expenses are covered up to ■1,00,000 per policy year when medically necessary and pre-authorised.

1.6 Home treatment

Domiciliary treatment is covered up to ■50,000 per policy year where the attending medical practitioner certifies that hospitalisation was not possible or would have been inappropriate.

2. Room Rent, ICU and Proportionate Deduction

2.1 Room category limit

Room rent is limited to **■5,000 per day**. ICU charges are separately limited to **■10,000 per day**. If the insured selects a room category above the eligible limit, the Company may apply a proportionate deduction to associated hospital expenses.

2.2 Proportionate deduction

Where the actual room rent exceeds the permitted room-rent limit, the ratio of the eligible room rent to the actual room rent may be applied to certain associated expenses, including nursing charges, procedure charges and professional fees, where such expenses are not separately itemised.

2.3 Example

If the eligible room rent is **■5,000 per day** and the selected room costs **■10,000 per day**, the Company may calculate certain associated charges using a 50% proportion. This example is illustrative and does not replace the claim assessment rules.

2.4 ICU

ICU charges above **■10,000 per day** are not payable. If ICU admission is accompanied by charges falling under another applicable sub-limit, the more restrictive applicable limit may apply.

3. Disease-Specific Limits

Treatment / Expense	Maximum payable
Cataract surgery	■40,000 per eye
Hernia surgery	■60,000
Joint replacement	■2,00,000 per joint
Maternity — normal delivery	■50,000
Maternity — caesarean	■75,000
Ambulance	■3,000 per admission
Home treatment	■50,000 per year

The above limits are part of the Sum Insured and are not additional benefits.

4. Waiting Periods and Pre-Existing Diseases

4.1 Initial waiting period

Except for accidental injuries, illnesses occurring during the first **30 days** from the commencement of the first Policy Period are not covered.

4.2 Specified disease waiting period

Claims relating to specified conditions including hernia, piles, sinusitis, cataract, benign prostatic hypertrophy, joint replacement and certain chronic disorders are payable only after completion of a **24-month** continuous waiting period.

4.3 Pre-existing disease

Pre-existing Diseases (PED) are covered only after completion of 36 months of continuous coverage, unless specifically waived in writing by the Company.

4.4 Definition

A Pre-existing Disease means a condition, ailment, injury or related condition for which the insured person received medical advice, diagnosis, treatment or medication during the 36 months before the commencement date.

4.5 Non-disclosure

Failure to disclose a known medical condition may result in claim rejection, policy cancellation or other action permitted under the Policy terms.

5. Co-payment and Deductible

5.1 Senior member co-payment

For an insured person aged 61 years or above, the insured shall bear a **20% co-payment** of the admissible claim amount, unless the schedule states a different percentage.

5.2 Voluntary deductible

A deductible of **■25,000 per policy year** applies where the selected plan schedule specifies the deductible option. The deductible is applied before the co-payment calculation.

5.3 Combined effect

Where both a deductible and co-payment apply, the deductible is first reduced from the admissible claim amount and the applicable co-payment is then calculated on the remaining amount.

6. Exclusions and Non-Medical Expenses

6.1 General exclusions

The following are not payable unless expressly covered by an endorsement: cosmetic treatment not medically necessary; experimental or unproven treatment; treatment arising from self-inflicted injury; non-prescribed supplements; and expenses solely related to comfort or convenience.

6.2 Non-medical expenses

Items such as registration charges, admission kit, visitor meals, toiletries, television charges, telephone charges, service charges, documentation charges and certain administrative consumables may be excluded even when incurred during an otherwise admissible hospitalisation.

6.3 Waiting-period exclusions

Expenses connected to conditions falling within an applicable waiting period remain excluded until that waiting period is completed, regardless of whether the hospitalisation itself occurs after the waiting period.

6.4 Alternative treatment

AYUSH treatment is covered up to █50,000 per policy year when provided at an eligible facility and subject to the terms applicable to the treatment category.

7. Claim Processing Charges

Charge	Amount / condition
Policy service request	█250 for selected non-mandatory requests
Duplicate policy document	█100
Cheque return	█500
Out-of-network reimbursement processing	No separate charge; subject to claim terms
Room upgrade	May trigger proportionate deduction

Charges may be revised at renewal subject to applicable notice requirements and the terms communicated by the Company.

8. Cashless and Reimbursement Claims

8.1 Cashless admission

Cashless treatment is available at participating network hospitals, subject to pre-authorisation and availability of the facility. Cashless approval does not by itself constitute final acceptance of every expense in the claim.

8.2 Reimbursement

For reimbursement claims, the insured should submit the claim form, original or acceptable electronic bills, discharge summary, prescriptions, investigation reports and other documents reasonably requested for claim assessment.

8.3 Claim intimation

Planned hospitalisation should normally be intimated at least 48 hours before admission. Emergency hospitalisation should be intimated as soon as reasonably possible and preferably within 24 hours.

8.4 Claim documents

Failure to submit required documents may delay assessment. Missing documents do not automatically establish that a claim is payable or not payable; the final decision depends on the Policy terms and available evidence.

8.5 Final assessment

The Company may assess the claim against coverage, exclusions, waiting periods, sub-limits, deductibles, co-payments, reasonable-and-customary expense requirements and medical necessity.

9. Renewal and Cancellation

9.1 Renewal

The policy may be renewed subject to the terms applicable at renewal and payment of the required premium.

9.2 Free-look period

A new policyholder may request cancellation during the applicable free-look period, subject to deductions permitted under the Policy and applicable rules.

9.3 Grace period

A grace period of 30 days may be available for renewal premium payment. Coverage during the grace period is subject to the terms stated in the Policy schedule.

10. Portability and Continuity

10.1 Portability

The insured may request portability to another health insurance product or insurer in accordance with applicable rules and the timelines communicated for portability.

10.2 Continuity credits

Eligible waiting-period credits may transfer subject to portability rules and the information available to the receiving insurer.

10.3 Break in coverage

A break in continuous coverage may affect waiting-period credits and eligibility for benefits. The precise consequence depends on the circumstances and applicable regulations.

11. Definitions

11.1 Reasonable and customary expenses

Reasonable and customary expenses are expenses for a medical service that are consistent with prevailing charges for similar treatment in the same geographical area, taking into account the nature and complexity of treatment.

11.2 Medically necessary

A treatment is medically necessary where it is required for diagnosis or treatment of the illness or injury, is not primarily for convenience, and is clinically appropriate for the insured person's condition.

11.3 Network hospital

A network hospital is a hospital that has an agreement or arrangement with the Company or its authorised administrator for providing eligible services under the cashless facility.

12. Important Clauses for Policyholders

1. A room upgrade can affect more than the room-rent component because proportionate deductions may apply.
2. A 36-month PED waiting period may materially delay coverage for pre-existing conditions.
3. A 20% senior-member co-payment can substantially increase the insured's out-of-pocket contribution.
4. Disease-specific sub-limits can restrict reimbursement even when the overall Sum Insured remains available.
5. Cashless approval should not be interpreted as final approval of every expense.

13. Synthetic Benchmark Data — For Testing the Compare Feature

The following table is deliberately included as a fictional benchmark section for testing only. A production system should obtain comparable figures from independently sourced, dated public documents rather than treating this table as regulatory evidence.

Attribute	This policy	Comparable A	Comparable B	Comparable C
Sum insured	■10 lakh	■10 lakh	■10 lakh	■15 lakh
Room rent	■5,000/day	■10,000/day	No fixed cap	■7,500/day
ICU	■10,000/day	■15,000/day	No fixed cap	■12,000/day
PED waiting	36 months	24 months	24 months	36 months
Specified disease	24 months	24 months	12 months	24 months
Senior co-pay	20%	10%	0%	10%
Deductible	■25,000	■10,000	■0	■20,000
Cataract sub-limit	■40,000/eye	■50,000/eye	■60,000/eye	■50,000/eye

14. Test Questions for Money Docs Decoded

Question 1: What happens if I choose a room costing ■8,000 per day?

Question 2: How long do I have to wait before a pre-existing disease is covered?

Question 3: What is the maximum cataract benefit?

Question 4: If the admissible claim is ■2,00,000 for a member subject to the 20% co-payment, how much would the co-payment be before considering any deductible?

Question 5: Which clauses could increase out-of-pocket expenses even when the policy has a ■10 lakh Sum Insured?

Question 6: Which clauses should be flagged as potential dispute-relevant patterns?

Question 7: Which policy attributes should be compared with external benchmark data?

Question 8: Give the exact page and clause supporting every answer.

End of synthetic policy.